

Heart Attack Proof! How To Prevent and Reverse Heart Disease

An Interview with Caldwell B. Esselstyn, M.D.

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KIRK HAMILTON: Welcome to **Staying Healthy Today**, a health-oriented radio show committed to bringing you key experts in the fields of nutrition, prevention and integrative medicine.

Hi, my name is Kirk Hamilton, your host of **Staying Healthy Today**, and our mission is simple: To provide you credible and usable health information from interviews and our educational resources to help you Stay and Be Well in the busy modern world. Please take a few moments before or after listening to this interview to browse through the **Prescription2000.com** website, the home of **Staying Healthy Today Radio**, for our free educational services.

Today's show topic is "**How To Become 'Heart Attack-Proof' Using Dr. Esselstyn's Program For Prevention And Reversal Of Heart Disease.**" Dr. Caldwell B. Esselstyn is our guest today. He is a graduate from Yale University in 1956, and he earned his M.D. degree from Western Reserve University School of Medicine in 1961. In 1956 he was part of the United States Rowing Team which was awarded a gold medal at the Olympic Games. In 1968 he was an Army surgeon in Vietnam where he was awarded the Bronze Star. Dr. Esselstyn has been associated with the world-renown Cleveland Clinic since 1968. He has more than 150 scientific publications. He is internationally known for his surgical expertise in endocrine and breast disease conditions. In 1995 he published his nutrition research showing arresting and reversal of coronary artery disease in severely ill patients. This same study was updated 12 years later making it one of the longest longitudinal studies of its type. No compliant patients have sustained disease progressions 20 years later.

So it's with my great pleasure I would like to welcome you, Dr. Esselstyn. Thank you so much for sharing your excellent work and not only on prevention but also on the reversal of heart disease.

DR. CALDWELL ESSELSTYN: Well thank you. I'm delighted to be with you.

KIRK HAMILTON: You know, I was thinking back, and so many times I've heard when people talk about when atherosclerosis starts, and we hear about the Korean (War) soldiers and when they did autopsies on them they had atherosclerosis present early on, and I kept thinking that obviously you were in Vietnam and you probably operated and saved a few lives doing some trauma surgery, or whatever type of surgery you did, and did it ever dawn on you that maybe those young boys had atherosclerosis and 40 years later you'd be at the forefront of prevention and reversal of this condition?

DR. CALDWELL ESSELSTYN: No, that certainly was not our mission at that time, but I would like to make one correction. I think it was while the Korean soldiers did occasionally have atherosclerosis it was actually kind of the other way around. When they did the autopsy studies it was found in 80% of our GIs at average age 20 already had gross evidence of atherosclerosis or heart disease that you could see without a microscope, whereas when you did the same on the Koreans or the Vietnamese it was only like 1% or 3%.

KIRK HAMILTON: Well thank you for correcting that. I really meant that. Tell me how does – how did your journey get to where it is? You are an internationally known thyroid and parathyroid and

breast cancer surgeon, and you did your surgery in Vietnam, and then you've come full circle and you're dealing with low-fat, plant-based diets to reverse atherosclerosis. How did that occur?

DR. CALDWELL ESSELSTYN: Well, actually six or eight years after I joined the clinic I was chairman of our breast cancer task force and we were very, very active but I became really quite disheartened and disillusioned with the fact that no matter how many women I was operating on for breast cancer I truly was not doing one single thing for the next unsuspecting victim. That led to a global sort of search of the literature and it was quite striking that in Kenya breast cancer was 20 times less frequent than in the United States and in rural Japan in the 1950s breast cancer was extremely infrequent. And yet when the Japanese women would migrate to the United States Japanese-Americans by the second and third generation now had the same rate of breast cancer as their Caucasian counterparts. Well so much for genes and even more compelling perhaps was cancer of the prostate which in the entire nation of Japan in 1958 - How many autopsy-proven deaths were there from cancer of the prostate? Eighteen! Eighteen in the entire nation! By 1978 they were now up to 237 which pales in comparison to the 28,000 who will die from cancer of the prostate this year in this country. So there was something that was very compelling that was going on and I perhaps made the mistake and assumption back in 1983, 84, that my bones would long be dust before I could get some answers with nutrition and cancer. In hindsight I'm not sure that's true, but nevertheless I therefore decided it would be more appropriate to look at the leading killer of women and men in Western civilization which is cardiovascular disease and heart attacks because in this global research it was very apparent that there were many cultures where heart disease and heart attacks were virtually nonexistent. I mean, even today if you're a cardiac surgeon and you go to rural China, or the Papua Highlands in New Guinea, or if you go to central Africa, or the Tarahumara Indians in northern Mexico, forget it. You'd better plan on selling pencils because you're not going to find cardiovascular disease there and it's largely because of that plant-based nutrition, and that's what made me feel that if we could get people to eat to save their heart they would actually then be saving themselves from cancer as well and I still think that's true.

KIRK HAMILTON: What are the lifestyle factors when people make that transition from the more traditional diets to the Western diet? What are the bad things that occur from our lifestyle that they incorporate?

DR. CALDWELL ESSELSTYN: Well, I think that the best way to answer that is to give you what I really call, and when I'm counseling patients, "throwing the hook." And you have to know that heart disease is truly nothing more than a 'toothless paper tiger' that need never ever exist, and if it does exist it need never ever progress. Now, how can I say that? You have to understand, and this is the key for any patient that we're counseling that all of our blood vessels are protected by this guardian or this lifejacket which are the endothelial cells, which are one layer thick and line all of our blood vessels. They are the absolute 'magic carpet' and they produce a substance which is nitric oxide, which does wonderful properties, keeps our blood flowing smoothly, keeps it from getting sticky, prevents inflammation from forming in the wall of the artery and prevents plaque from forming, and it really can destroy some of those inflammatory cells so that's the key. And there has been some wonderful investigative work just in the last decade or so that clearly shows every time that you even - this is with young subjects using what we call a brachial artery tourniquet test, we can show that literally within minutes of eating oil, dairy and meat that you damage and injure and insult the endothelial cells so that it immediately diminishes its capacity to make this magical nitric oxide. So you can imagine that in the typical Western diet when you're 14 years of age if you were to spread out all your endothelial cells you could cover six or eight tennis courts. However, as you progress through adolescence and young adulthood and continue to assault and injure these endothelial cells then by your 40s and 50s, then you've really got enough disease to begin to have heart attacks and strokes.

KIRK HAMILTON: So it's safe to say that the dietary changes of the Western world, an increase in total meat consumption, added fats and oils and dairy products are a significant reason –

DR. CALDWELL ESSELSTYN: Oh they're lethal! We now know with every meal you're damaging it. It's no longer a secret. It's been study after study that it's showed that. And that's why we got such striking results, because although I'm not quite as mean as I look, if you look at the cover of my book, **"Prevent and Reverse Heart Disease,"** you'll find my picture – although I'm not as mean as I look, I am a taskmaster! I have no interest in trying to counsel patients who don't have the same shared vision that I have for them, which is not to slow the rate of disease progression but we have to *'annihilate'* this disease and reverse it.

KIRK HAMILTON: So let me ask then, obviously added fats and oils aren't great. What is your commentary on nuts and seeds in their whole state since there are studies which show that they help reduce heart attack incidence?

DR. CALDWELL ESSELSTYN: I don't know of a good study yet with nuts that hasn't been funded by the nut companies, and they often show what we call surrogate biomarkers, and you're supposed to assume that because you eat nuts and it might change your HDL cholesterol to a better pattern, or your LDL to a better pattern, that that's going to translate into less disease. I'm not convinced that that's really been shown because if I ever go on record as saying nuts are okay, and remember now nuts contain a lot of saturated fat which is the absolute building block for heart disease. If you say "If Esselstyn says that nuts are okay," suddenly then where are the nuts? They're in the glove compartment, they're at the work bench, they're in the entry to the house, they're in the bedroom, they're in the kitchen, they're even in the bathroom. When you say nuts are okay, nuts are very, very addicting. They don't just have a little handful once every few days. So you're pouring in all this saturated fat. I want to see the study that shows me that they can show the reversal of the disease.

KIRK HAMILTON: With nut consumption....I got it. Well how about let's talk about cholesterol. The buzzword. How low do you drive cholesterol down? How important do you think that is or is that a surrogate marker for atherosclerosis?

DR. CALDWELL ESSELSTYN: I think cholesterol is important, but you have to recall that cholesterol cannot really be separated from the importance of what somebody eats. Let's say that somebody is eating the typical toxic Western diet full of saturated dairy fat and meat and oils, and yet if they take a 'gorilla' dose of a statin they can block an essential enzyme in cholesterol manufacture in their liver so that when they have their cholesterol checked their doctor will say, well that's a good number. Well then they continue to eat this way. Well that doesn't protect you. That doesn't really protect you at all because what you're doing is you are still assaulting, and you're still injuring the lifejacket of your blood vessel, those endothelial cells, and as the nitric oxide is diminished more and more that drug with the low cholesterol is not protecting you. The disease marches on. So I think it's wonderful if you just, for instance, get a magnificent cholesterol reading by just changing your diet alone or – I'm not unwilling to accept the fact that somebody takes a low dose of a statin, but I'm a taskmaster when it comes to nutrition because that's what seems to trump it all.

KIRK HAMILTON: Tell me about your dairy product rap. I continue to get in 'fights' with patients, and for years I've been taking people off dairy products and sometimes they're just dumbfounded when they look at me when I say get off dairy products. Give me your 'two cents'?

DR. CALDWELL ESSELSTYN: Well, I think there's been wonderful work by Steve Nichols from Adelaide in Australia. He did brilliant work showing that literally within, again within minutes of you

eating or ingesting heavy milk products loaded with saturated fat, that the endothelial cell's capacity to manufacture nitric oxide immediately is assaulted. And so there's no escaping the damage that dairy products do.

KIRK HAMILTON: You've had serious patients. How fast can you get their atherosclerosis to reverse, and how sick are they when they start?

DR. CALDWELL ESSELSTYN: Our original group of patients were literally, as my late brother-in-law used to call them, they were the 'walking dead.' I mean these were patients that failed their first or second bypass, they failed their first or second angioplasty, they were too sick for these procedures or they had refused them. And what is quite striking is when I – and there were five in that group that were told by expert cardiologists they wouldn't live out the year. And you already mentioned, all five of those patients and all the others fully compliant went well beyond 22, 23 years. And as far as the speed of this is concerned, that's really quite exciting. We have been able to show that as early as three weeks that the endothelial cells have sufficiently recovered so they can once again dilate even a diseased vessel enough so that the amount of blood going to the heart muscle is markedly increased within three weeks, but it takes an additional number of months and that varies somewhere between a year to sometimes 30 months. In 30 months we've had it completely disappear. This is in a person who was in their 40s and it took him 40 years to practically block the heart artery and yet in 30 months he was able to open it.

KIRK HAMILTON: Is there such a thing as too low a fat diet in your opinion?

DR. CALDWELL ESSELSTYN: We haven't strived to make this a low fat diet. What we try to emphasize is that this is plant-based nutrition. And when you are eating a diet that is primarily composed of all the whole grains for your cereal, bread and pasta, '101' different types of legumes and beans, the vegetables red, yellow and green, and fruit, that combination when it's eaten will be somewhere around 11, 12% fat which is far and away enough to take care of all the essential fat, and at the same time it really will get the job done as far as halting this disease and setting you on the path to reversal. There's one thing that I really have to mention, and that is if you think of when a patient comes to see me, if you will think of their blood vessels as a brush fire, or a house on fire, there are two ways you stop the fire. One is you absolutely make it clear that never again to pass their lips are any of those foods that are going to continue to stoke the fire or pour gasoline on it, which are the oils, the meat, the dairy, the nuts and the avocado. And the other thing that has to be done to pour water on the fire is as wonderful as the green leafy vegetables, and by that I mean the ones that have all those natural phytonutrients, bioflavonoids and antioxidants which are so essential to quell the inflammation. That's the bok choy, swiss chard, kale, collards, collard greens, beet greens, mustard greens, turnip greens, Napa cabbage, brussel sprouts, broccoli, cauliflower, cilantro, parsley, spinach and arugula, just as a sample.

KIRK HAMILTON: You've said that probably once or twice, and I'm laughing because my – the producer has me do a 'Peter, Piper, Picked a Pepper...', and if I can get that one when I transcribe it, I'm going to do that for my practice (sound test). That was really, really good.

We're talking to Dr. Caldwell B. Esselstyn, and his excellent book is "**Prevent and Reverse Heart Disease**" and he says you can become 'heart-attack proof.'

Tell me, how do people convert to your program? Are they kicking and screaming, or do you just get the committed ones to jump and do it?

DR. CALDWELL ESSELSTYN: Well, I think what has happened is that people – of course we do see people who say they have a strong family history, they never want to have the disease, and those of course

are one set of patients that we see. The others are those that say, “My gosh! I’ve been told that I have these blockages but I refused to have that stent. I just don’t want to go through that and all the complications or I refuse to take the bypass surgery. I want to give this a try first.” And those of course are extremely committed patients and they absolutely rejoice. When they fully appreciate and recognize that they, not their cardiologist or their heart surgeon, that they themselves now have become the absolute locus of control for annihilating this disease....This is not cancer.

KIRK HAMILTON: I hear your passion and I hear your clarity because you know that this occurs. You know we have a healthcare debate right now. I was going to save this question until the end, but we have a healthcare debate. We’re spending billions upon billions of dollars and it doesn’t really matter what kind of healthcare system we have if you don’t prevent. So what would be your four or five suggestions to..... if you could stand up in front of Congress and be – or be a ‘benevolent dictator’ for about a month?

DR. CALDWELL ESSELSTYN: Well, I think the first thing we would want to do is show that we have great respect for any patient that we see and also if we were some sort of dictator we’d have respect for the public. I would try to have them first of all, I would try to have them have the following nuggets and piece of knowledge. You wouldn’t dictate to them. You would give them the knowledge. You’d let them know that the end result of what I’m about to say, *I would paraphrase John Kennedy. And that is to say, “Ask not what your country can do for your health. Ask what you can do for your health.” And by that I mean that we have now got pretty solid documentation that about 80% of these common chronic killing diseases that afflict this country really need never exist and are entirely related to lifestyle. And by that, once again food trumps it all. Yes exercise is important, and to have good social interaction is important, but I don’t care how wonderful your friends are and how often you run if you are eating the wrong foods you’re done. You’re finished! Both in terms of cancer and heart disease.* And let me explain why it is that I think we are on the absolute cusp of what could be a magnificent health revolution. I’m going to take the position now that we don’t have multiple chronic killing diseases. What we really have is one. And let me explain. Let’s suppose that I’ve got a patient who weighs 240 pounds who’s had a heart attack and he or she is counseled and they get it. Now remember, when they’re 240 pounds they are obese, they are hypertensive, they are at risk for stroke, they are also diabetic, and they get it. They just absolutely get it. Now it’s six or seven months later and they weigh about 180, 185. They really are no longer obese. They are no longer diabetic. They are no longer hypertensive. They are certainly not at risk for another heart attack or stroke providing that they are eating the way that we’ve instructed them. They also markedly diminished the likelihood of ever acquiring one of the common Western cancers of breast, prostate, colon, endometrial, ovary and even pancreatic. They’re not gonna get gallstones, they’re not gonna get diverticulitis. They’re not gonna get osteoporosis. They’re not gonna get rheumatoid arthritis, asthma, dementia, erectile dysfunction and Alzheimer’s. I mean these things are all compounded by food, and it’s so incredibly powerful what this mission can be so that would probably be my message is to try to get people to be educated. Imagine. The thing that would really tip the scales. Imagine how many men in this country would continue to eat dairy, oil and meat if they know that this was absolutely hastening them on the path of erectile dysfunction. Sixty million men in this country have erectile dysfunction. Now, my gosh, why in the world would they want to let that happen? The vessels that go to the penis are so much smaller than the coronary arteries. Long before somebody has symptoms of heart disease they start having problems with erectile dysfunction, and rather than reach for Viagra or Cialis, they really should see their cardiologist or they should change their lifestyle. We’ve had many people after we’ve counseled them - I’ve had men call me six or eight months later and say, “Dr. Esselstyn, I think I really owe you another check!”

KIRK HAMILTON: Well you know that answer was so good, and you’re preaching to the choir here, and eloquently said is that this lifestyle change would obviously impact the major chronic diseases

that are plaquing Western society.

I have a question for you being a former world class athlete and a rower. How do you think a plant-based diet works in an athlete? Not, you know, a weekend warrior but also in an elite athlete?

DR. CALDWELL ESSELSTYN: Sure. My son was a professional triathlete for 10 years before he became a fireman and he actually has written a book called “**The Engine 2 Diet.**” Rip Esselstyn, and it didn’t slow him down any but before he wrote his book last year, let’s see, he was 46 and he decided that he’d better enter the United States Master’s Swimming Championships. Now when he was at Texas he was an All-American swimmer but there were people who were beating him certainly in the backstroke from time to time and now 23 years later here in that age group that he entered in between 45 and 50 he was up against these same guys that had regularly had a chance to beat him when they were in college. And it was a 200 yard backstroke and off they went in the race and they were ahead of him for the first 100 yards and the second 100 yards he came down and he absolutely crushed them and he set a new Master’s swimming record for his age group, breaking the old record by over 2 seconds. And I maintain that the reason that Rip was able to do that was for the last 23 years while his competitors have been trashing their endothelial cells in their body Rip has actually been enhancing his by his plant-based type of nutrition. Tony Gonzales is another example of a professional athlete who is fully committed to plant-based nutrition. You know new records with touch-downs, yards gained and the number of catches. There are other professional athletes like Carl Lewis who was plant-based and perhaps the strongest one of all was Dave Scott who six years running won the Hawaii Ironman.

KIRK HAMILTON: Actually, interesting enough, it’s full circle. When I was playing football in college at UC Davis, Dave Scott was this guy in the gym that would go on forever and I never knew who he was until I saw him a couple years later as the triathlon ‘king’.

In closing, I had a couple of questions. Do you recommend supplements at all?

DR. CALDWELL ESSELSTYN: Vitamin D3, some calcium and certainly B12.

KIRK HAMILTON: And lastly what’s your comment on soy? There’s a lot of controversy on soy. Do you think it’s good or bad?

DR. CALDWELL ESSELSTYN: Oh I think that – you know we’ve got a couple of recipes for soy. I don’t think it’s good to go overboard on soy and especially I don’t like it if it’s highly processed soy. But soy has an awful lot of fat . Soy is 40% fat so we’re not too much on fat and I think there may be some down side to overdosing on soy protein. My friend Collin Campbell the nutritionist from Cornell has pretty well convinced me that excessive soy leads to an increase in our IGF which is our insulin growth factor which is a tumor promoter.

KIRK HAMILTON: Tell me how – or tell the audience how people can get a hold of your book and get connected with your work.

DR. CALDWELL ESSELSTYN: Probably get it through Amazon or Borders or Barnes and Noble.

KIRK HAMILTON: Do you have a website called **Heartattackproof?** Is that correct?

DR. CALDWELL ESSELSTYN: **www.Heartattackproof.com.** You’re correct. Yes.

KIRK HAMILTON: That's a pretty progressive statement. That must get your colleagues at the Cleveland Clinic excited.

DR. CALDWELL ESSELSTYN: Nothing wrong with telling it like it is.

KIRK HAMILTON: Well, I want to thank you very much for your great work and dedication and sharing so clearly and forcefully that not only can you prevent heart disease but you can reverse the number one killer which is heart disease. And I want to thank you, the audience, for listening today on this edition of **Staying Healthy Today Radio**. And remember until next time Stay and Be Well.

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